

Supplemental Online Content

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This supplemental material has been provided by the authors to give readers additional information about their work.

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eMethods

Dose Titration in FIND-CKD

Finerenone was initiated at 10 mg once daily in patients with estimated glomerular filtration rate (eGFR) between ≥ 25 and < 60 mL/min/1.73 m² at screening, and at the higher (target) dose of 20 mg once daily in patients with eGFR ≥ 60 mL/min/1.73 m². Patients who started on the lower 10 mg dose were up-titrated to the target dose from month 1 if serum potassium was ≤ 4.8 mmol/L and eGFR decrease was less than 30% of the last measured value. Down-titration or interruption of study drug was permitted at any time for safety reasons. Interruption was mandated if serum potassium was > 5.5 mmol/L.

Statistical Analysis

For eGFR slope analysis, in addition to treatment-specific fixed effects (finerenone vs placebo) for the intercept, time from randomization, and time difference relative to the change point, the model included fixed effects for the stratification factors: baseline sodium glucose cotransporter-2 inhibitor (SGLT2i) use (yes/no) and screening urinary albumin to creatinine ratio (UACR; ≤ 1000 mg/g versus > 1000 mg/g). The model also included interactions between these fixed effects and the time components. Random effects for the intercept, acute slope (baseline to month 3), and directional change in slope at the change point (month 3) were included to account for variation in patients' progression rates over time. The unstructured covariance matrix of the random effects was allowed to vary freely between treatments.

For the analysis of UACR and urinary protein-to-creatinine ratio change over time, the model included factors for the treatment group, visit, treatment-by-visit interaction, baseline SGLT2i use, the interaction between SGLT2i use and visit, log-transformed baseline UACR value, and an interaction term between the log-transformed baseline value and visit.

Imputation of Intercurrent Events

The composite strategy for the intercurrent events dialysis or kidney transplant and death in the main analytical approach for the primary efficacy outcome (total slope at month 32) was implemented with a resampling imputation procedure, using the following prespecified steps: 1) The earliest occurrence of death or dialysis or kidney transplant was identified for each participant; 2) For eGFR assessments expected under the primary estimate within the planned treatment period and those that occur after this time point, the analysis value was treated as missing and imputed using multiple imputation; 3) Imputed values were generated by resampling with replacement from an empirical donor pool of clinically unfavorable eGFR values observed in this study, defined using eGFR derived from the 2009 Chronic Kidney Disease Epidemiology Collaboration (CKD-EPI) creatinine equation. This involved identifying potential donors as participants with dialysis or kidney transplant in the full analysis set. For each potential donor, the last available eGFR measurement prior to the onset of dialysis or kidney transplant (based on either central or local laboratory) was derived. If the value above was 15 mL/min/1.73 m² or less, it was included in the donor pool; otherwise, it was removed. Each missing assessment due to dialysis or kidney transplant and death was drawn with replacement from this donor pool; and 4) 50 completed datasets were generated, analyzed using the prespecified primary model, and combined using Rubin's rules.

Supplementary Tables

eTable 1. FIND-CKD Full Inclusion and Exclusion Criteria

Inclusion criteria	Exclusion criteria
≥18 years of age	Type 1 or 2 diabetes mellitus, or HbA1c ≥ 6.5% (48 mmol/mol)
Clinical diagnosis of CKD and: - eGFR ≥25–<60 mL/min/1.73 m² and UACR ≥200–<500 mg/g^a or - eGFR ≥25–<90 mL/min/1.73 m² and UACR ≥500–≤3500 mg/g - Documentation of albuminuria/proteinuria^b in medical records at least 3 months prior to screening	Systolic blood pressure ≥160 mm Hg or diastolic blood pressure ≥100 mm Hg at screening
Stable and maximum tolerated labeled dose of an ACEi or ARB for at least 4 weeks prior to screening	Autosomal dominant or autosomal recessive polycystic kidney disease
Serum potassium ≤4.8 mmol/L at screening	Lupus nephritis or ANCA-associated vasculitis or any other primary or secondary kidney disease requiring immunosuppressive therapy within 6 months prior to screening
Negative pregnancy test at screening and agreement to use adequate contraception among women of childbearing potential	History of organ transplantation
Capable of giving signed informed consent	Acute kidney injury requiring dialysis within 6 months prior to screening
	Symptomatic heart failure with reduced ejection fraction with class 1A indication for an MRA
	UACR >3500 mg/g at screening ^c
	Cardiovascular event ^d within 3 months prior to screening or elective percutaneous coronary intervention within 1 month prior to screening
	Known hypersensitivity to the study treatment (active substance or excipients)
	Addison's disease
	Hepatic insufficiency classified as Child-Pugh C
	Concomitant therapy with: Eplerenone, spironolactone, esaxerenone, any renin inhibitor, sacubitril/valsartan combination, or potassium-sparing diuretic that cannot

	be discontinued at least 4 weeks prior to screening • Both ACEi and ARB if one of those cannot be discontinued at least 4 weeks prior to screening • Potent CYP3A4 inhibitors or moderate and potent inducers (to be stopped at least 7 days before randomization)
	Prior/concurrent clinical study experience: • Previous assignment to treatment during this study • Simultaneous participation in another interventional clinical study or treatment with any investigational medicinal product within 30 days prior to randomization
	Other: • Any other condition or therapy, which would make the participant unsuitable and would not allow participation for the full planned study period (e.g. active malignancy or other condition limiting life expectancy to less than 12 months) • Any other history, condition, therapy, or uncontrolled intercurrent illness that could in the opinion of the investigator affect compliance with study requirements • Pregnant or breast-feeding or intention to become pregnant during the study • Close affiliation with the investigational site; e.g. a close relative of the investigator, dependent person (e.g. employee or student of the investigational site)

^aCapped at approximately 10% of the total population.

^bQuantitative or semiquantitative measurement documented in the medical records.

^cOne reassessment was allowed if UACR was >3500 mg/g in one of the three urine samples collected at the screening visit.

^dHeart failure decompensation, myocardial infarction, stroke, transient ischemic attack, pulmonary embolism, elective coronary artery bypass grafting.

Abbreviations: ACEi, angiotensin-converting enzyme inhibitor; ANCA, antineutrophilic cytoplasmic autoantibody; ARB, angiotensin receptor blocker; CYP3A4, cytochrome P450 3A4; eGFR, estimated glomerular filtration rate; HbA1c, glycated hemoglobin; MRA, mineralocorticoid receptor antagonist; UACR, urinary albumin-to-creatinine ratio.

eTable 2. Demographic and Clinical Characteristics of the Overall FIND-CKD Population and Participants With Glomerular Diseases at Baseline

Characteristic	Overall FIND-CKD population (N = 1584)	With glomerular diseases (N = 903)
Age, y, mean (SD)	54.7 (14.3)	51.1 (13.7)
Sex, No. (%)		
Female	535 (33.8)	362 (40.1)
Male	1049 (66.2)	541 (59.9)
Race, No. (%)		
White	648/1578 (41.1)	314/901 (34.9)
Black	37/1578 (2.3)	13/901 (1.4)
Asian	868/1578 (55.0)	558/901 (61.9)
Other ^a	25/1578 (1.6)	16/901 (1.8)
Medical history, No. (%)		
Hypertension	1407 (88.8)	755 (83.6)
Atherosclerotic disease	195 (12.3)	63 (7.0)
Heart failure	35 (2.2)	10 (1.1)
Primary cause of CKD, No. (%)		
Hypertensive/ischemic nephropathy	459 (29.0)	0
Glomerular disease	903 (57.0)	903 (100.0)
IgA nephropathy	416 (26.3)	416 (46.1)
Focal segmental glomerulosclerosis	215 (13.6)	215 (23.8)
Other chronic glomerular diseases ^b	156 (9.8)	156 (17.3)
Membranous nephropathy	90 (5.7)	90 (10.0)
Membranoproliferative glomerulonephritis	26 (1.6)	26 (2.9)
Other ^c	53 (3.3)	0
Unknown	169 (10.7)	0
CKD etiology confirmed by biopsy	N/A	712 (78.8)
Medication use, No. (%)		
SGLT2i	270 (17.0)	190 (21.0)
Diuretics	290 (18.3)	123 (13.6)
Clinical measurements: mean (SD)		
BMI, kg/m ²	27.6 (5.6)	27.0 (5.5)
Systolic blood pressure, mm Hg	129.5 (14.1)	127.1 (13.8)
Biochemical measurements		
UACR, mg/g, median (q1, q3)	818.9 (577.4, 1244.0)	839.6 (587.3, 1255.1)
UACR category, mg/g, No. (%)		
<300	62 (3.9)	25 (2.8)
300-1000	930 (58.7)	533 (59.0)
>1000	592 (37.4)	345 (38.2)
eGFR, mL/min/1.73 m ² , mean (SD)	46.7 (16.1)	48.8 (16.7)
eGFR category, mL/min/1.73 m ² , No. (%)		
<45	844 (53.3)	431 (47.7)
45-<60	423 (26.7)	251 (27.8)
≥60	317 (20.0)	221 (24.5)
UPCR, median (min, max), mg/g	N/A	1226.6 (171.0-6313.2)

^aIncluded American Indian or Alaska native, native Hawaiian or other Pacific islander or multiple. The multiple-race category included participants who reported belonging to more than one racial group.

^bIncluded any non-pre-specified terms.

^cPrimary cause of CKD not classified as chronic glomerulonephritis and not coded as either “chronic kidney disease” or “nephropathy” based on medical history directory terms.

Abbreviations: BMI, body mass index; CKD chronic kidney disease; CRP, C-reactive protein; eGFR estimated glomerular filtration rate; IgA, immunoglobulin A; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UACR urinary albumin-to-creatinine ratio; UPCR, urinary protein-to-creatinine ratio.

eTable 3. Total eGFR Slope at 32 Months According to Baseline Albuminuria in Participants with Glomerular Diseases

Outcome	Finerenone n=446	Placebo n=457	Treatment difference	<i>P</i> _{Interaction}
eGFR slope, mL/min/1.73m²/year (95% CI)				
Overall	-3.50 (-3.85 to -3.15)	-4.23 (-4.59 to -3.86)	0.73 (0.22 to 1.24)	
According to baseline UACR				
≤1000 mg/g	-2.85 (-3.30 to -2.40)	-3.30 (-3.77 to -2.84)	0.46 (-0.19 to 1.10)	0.1834
>1000 mg/g	-4.53 (-5.09 to -3.97)	-5.69 (-6.29 to -5.10)	1.17 (0.34 to 1.99)	

Mean rate of change in eGFR from baseline to month 32 (total slope) calculated using a 2-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization, and time difference relative to the change point (month 3); baseline SGLT2i use; screening UACR category; and interactions of SGLT2i use and UACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. Intercurrent events dialysis or kidney transplant and death were handled via a composite strategy in a multiple imputation approach, by drawing multiple times with replacement from a pool of unfavorable eGFR values, consisting of all eGFR values <15 mL/min/1.73 m² collected as last values before onset dialysis or kidney transplant in the study.

Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UACR, urinary albumin-to-creatinine ratio.

eTable 4. Summary of Participants With Glomerular Diseases With Treatment-Emergent AEs

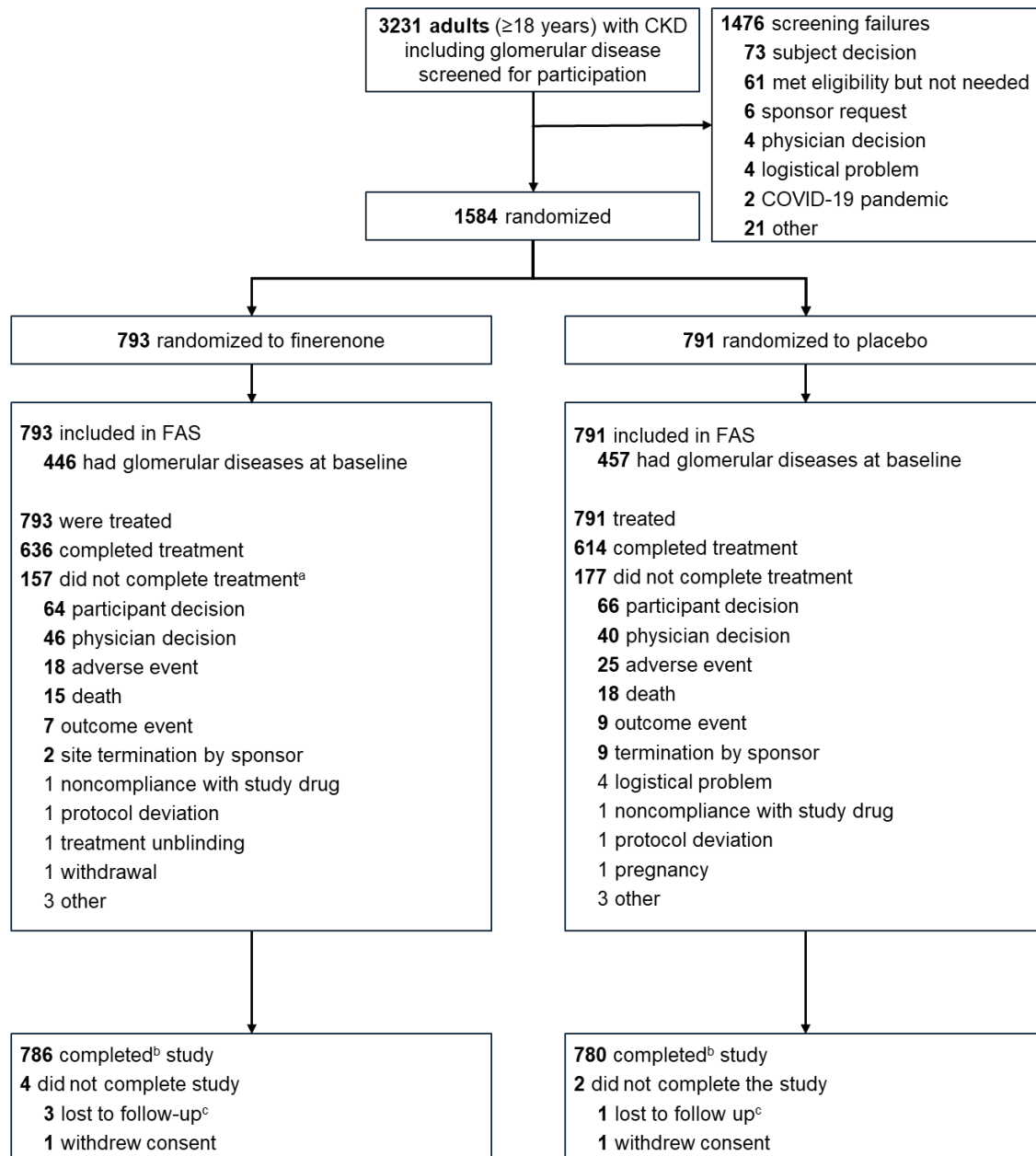
AE. No. (%)	Finerenone (n = 446)	Placebo (n = 457)
Any AE	306 (68.6)	300 (65.6)
Related to study drug	177 (26.2)	86 (18.8)
Leading to permanent discontinuation of study drug	12 (2.7)	18 (3.9)
Any SAE	87 (19.5)	98 (21.4)
Related to study drug	4 (0.9)	4 (0.9)
Leading to permanent discontinuation of study drug	1 (0.2)	5 (1.1)
Leading to hospitalization	85 (19.1)	94 (20.6)
Life-threatening	2 (0.4)	5 (1.1)
Leading to death	2 (0.4)	1 (0.2)
Any hyperkalemia (AESI) ^a	73 (16.4)	70 (15.3)
Related to study drug	54 (12.1)	49 (10.7)
Leading to permanent discontinuation of study drug	8 (1.8)	2 (0.4)
Any serious hyperkalemia	4 (0.9)	4 (0.9)
Related to study drug	2 (0.4)	1 (0.2)
Leading to hospitalization	3 (0.7)	4 (0.9)
Life-threatening	1 (0.2) ^a	0
Leading to death	0	0

^aHyperkalemia was an AESI and defined as any investigator-reported AEs with MedDRA codes corresponding to the preferred terms hyperkalemia or blood potassium increased.

Abbreviations: AE, adverse event; AESI, adverse event of special interest; MedDRA, Medical Dictionary for Regulatory Activities; SAE, serious adverse event.

Supplementary Figures

eFigure1. CONSORT Diagram (Full FIND-CKD Population)



At month 32, 494 (81%) participants in the finerenone group were treated with a daily 20 mg dose and in the placebo group, 535 (88%) received matching placebo treatment.

^aTwo participants in the finerenone group discontinued treatment for more than one reason.

^bParticipants were considered to have completed the study if there was contact with the participant after the end-of-study notification or if the subject died.

^cExcluding participants in Russia (n = 3 in the finerenone group, n = 9 in the placebo group; all completed the month 32 visit), who were administratively discontinued from the study and all electronic systems closed in Q1/25 to comply with company policy; no further follow-up was possible.

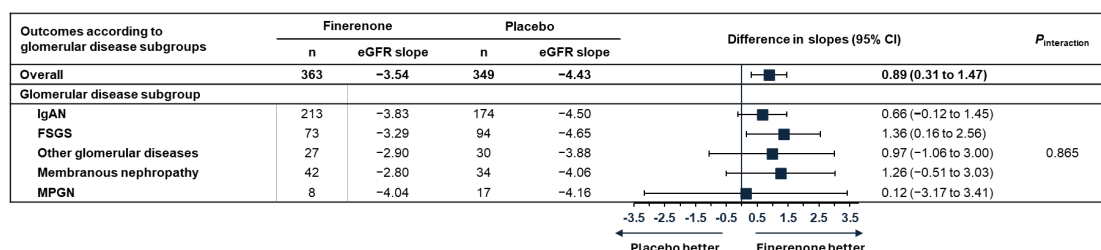
Abbreviations: CKD, chronic kidney disease; COVID-19, coronavirus 2019; FAS, full analysis set.

Adapted with permission from Heerspink HJL, et al. Finerenone in people with chronic kidney disease. *N Engl J Med*.

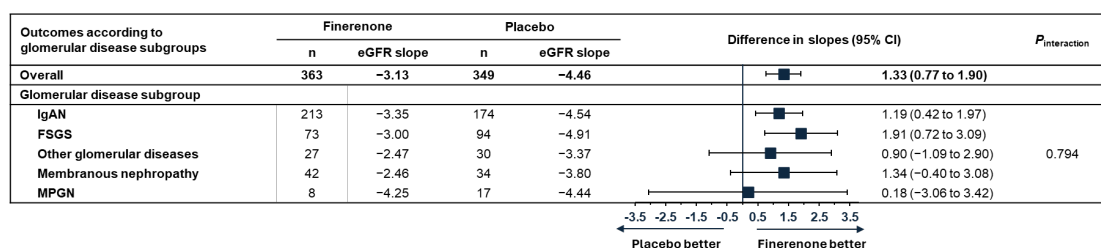
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eFigure2. Subgroup Analysis According to Glomerular Disease Etiology (Confirmed with Biopsy) for (A) Change in eGFR From Baseline to Month 32 (Total Slope), (B) Change in eGFR From Month 3 to End of Treatment (Chronic Slope), (C) Change in UACR From Baseline to Month 12

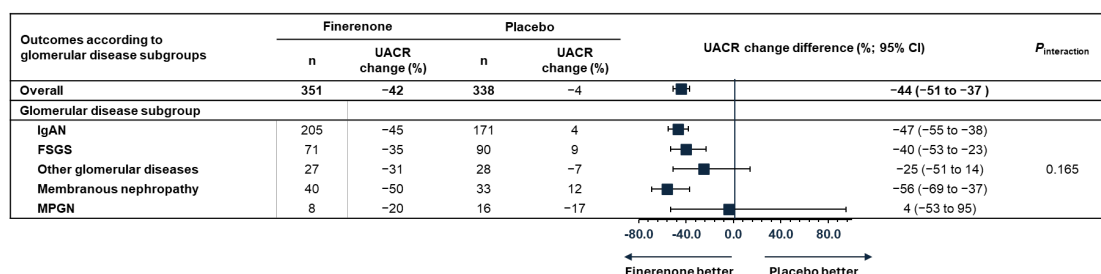
A. Change in eGFR from Baseline to Month 32 (total slope) in biopsy-confirmed participants



B. Change in eGFR from Month 3 to End of Treatment (chronic slope) in biopsy-confirmed participants



C. Change in UACR from Baseline to Month 12 in biopsy-confirmed participants



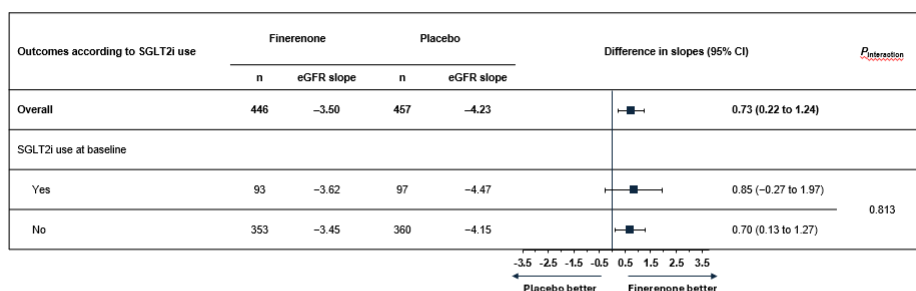
(A) Mean rate of change in eGFR from baseline to month 32 (total slope) and (B) from month 3 to end of treatment (chronic slope) calculated using a 2-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization, and time difference relative to the change point (month 3); baseline SGLT2i use; screening UACR category; and interactions of SGLT2i use and UACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. Intercurrent events dialysis or kidney transplant and death were handled via a composite strategy in a multiple imputation approach, by drawing multiple times with replacement from a pool of unfavorable eGFR values, consisting of all eGFR values <15 mL/min/1.73 m² collected as last values before onset of dialysis or kidney transplant in the study. The square represents the estimate of the total slope difference at 32 months; the lower and upper ends of the whiskers indicate the limits of the 95% CI.

(C) UACR analyzed using a mixed model for repeated measures with fixed effects for the following factors: treatment group, visit, treatment by visit interaction, baseline SGLT2i use, and the interaction between SGLT2i use and visit, with the log-transformed baseline UACR value as a covariate and an interaction term between the log-transformed baseline value and visit. The square represents the estimate of the UACR percentage change to placebo from baseline to month 12; the lower and upper ends of the whiskers indicate the limits of the 95% CI.

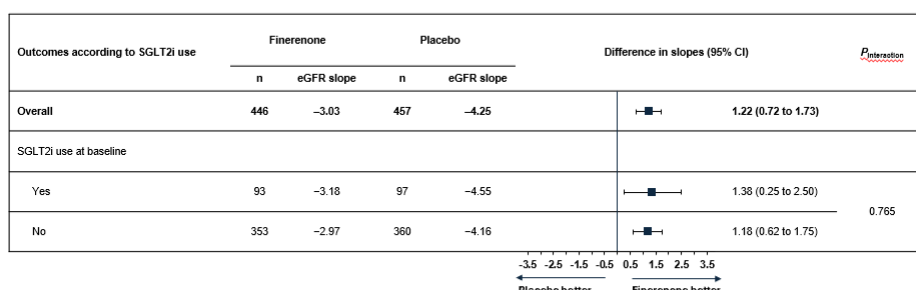
Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; FSGS, focal segmental glomerulosclerosis; IgAN, immunoglobulin A nephropathy; MPGN, membranoproliferative glomerulonephritis; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UACR, urinary albumin-to-creatinine ratio.

eFigure3. Subgroup Analysis According to SGLT2i Use at Baseline for A) Change in eGFR From Baseline to Month 32 (Total Slope), (B) Change in eGFR From Month 3 to End of Treatment (Chronic Slope), (C) Change in UACR From Baseline to Month 12

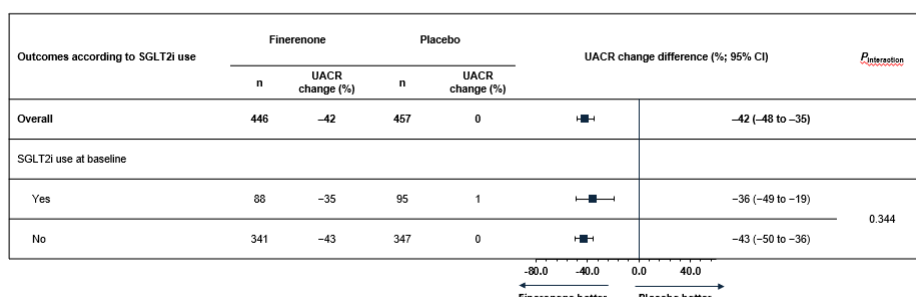
A. Change in eGFR from Baseline to Month 32 (total slope)



B. Change in eGFR from Month 3 to End of Treatment (chronic slope)



C. Change in UACR from Baseline to Month 12

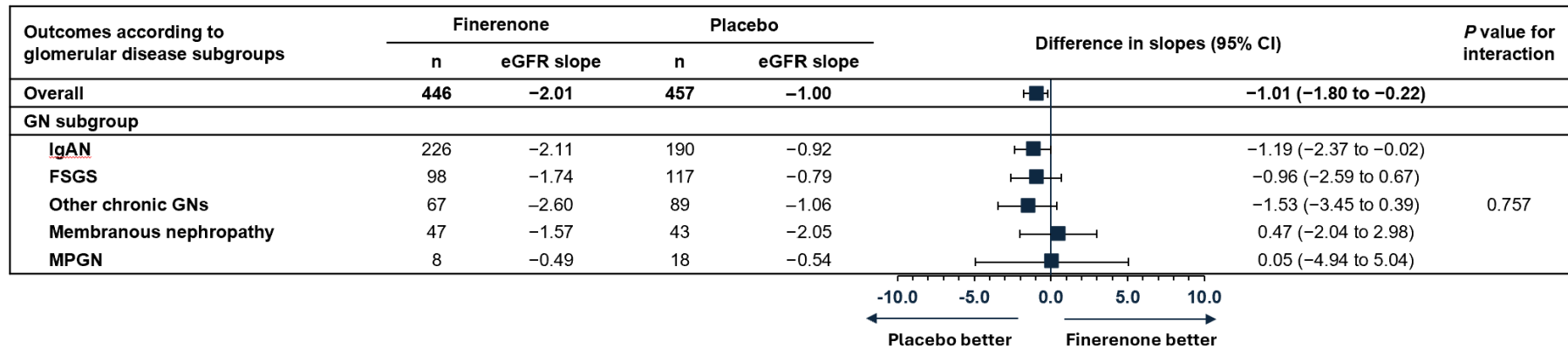


(A) Mean rate of change in eGFR from baseline to month 32 (total slope) and (B) from month 3 to end of treatment (chronic slope) calculated using a 2-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization, and time difference relative to the change point (month 3); baseline SGLT2i use; screening UACR category; and interactions of SGLT2i use and UACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. Intercurrent events dialysis or kidney transplant and death were handled via a composite strategy in a multiple imputation approach, by drawing multiple times with replacement from a pool of unfavorable eGFR values, consisting of all eGFR values <15 mL/min/1.73 m² collected as last values before onset of dialysis or kidney transplant in the study. The square represents the estimate of the total slope difference at 32 months; the lower and upper ends of the whiskers indicate the limits of the 95% CI.

(C) UACR analyzed using a mixed model for repeated measures with fixed effects for the following factors: treatment group, visit, treatment by visit interaction, baseline SGLT2i use, and the interaction between SGLT2i use and visit, with the log-transformed baseline UACR value as a covariate and an interaction term between the log-transformed baseline value and visit. The square represents the estimate of the UACR percentage change to placebo from baseline to month 12; the lower and upper ends of the whiskers indicate the limits of the 95% CI.

Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UACR, urinary albumin-to-creatinine ratio.

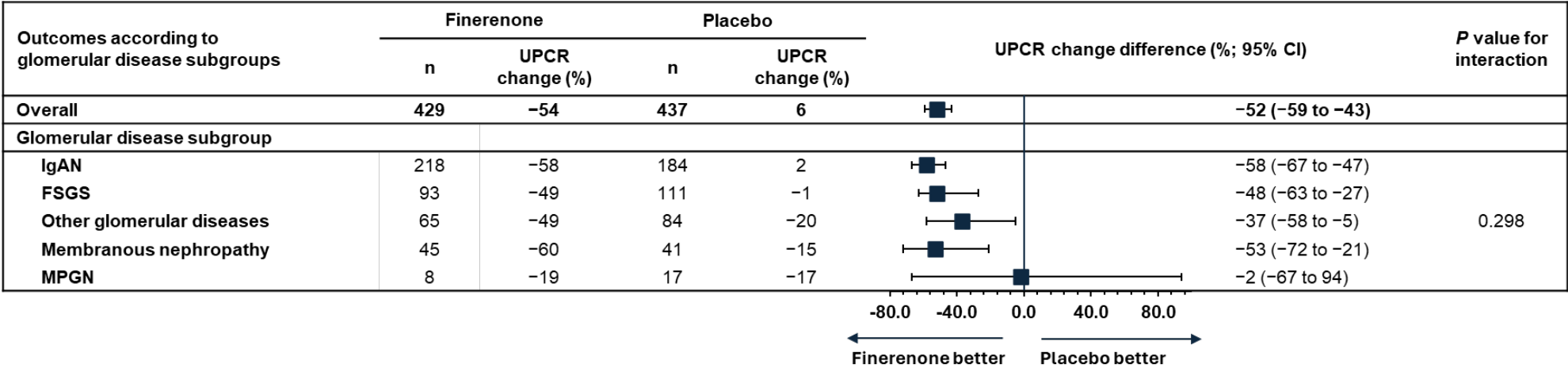
eFigure4. Change in eGFR From Baseline to Month 3 (Acute Slope) According to Glomerular Disease Etiology



Annualized mean rate of change in eGFR from randomization to month 3 calculated using a 2-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization and time difference relative to the change point (month 3); baseline SGLT2i use; screening UACR category; and interactions of SGLT2i use and UACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. Intercurrent events dialysis or kidney transplant and death were handled via a composite strategy in a multiple imputation approach, by drawing multiple times with replacement from a pool of unfavorable eGFR values, consisting of all eGFR values <15 mL/min/1.73 m² collected as last values before onset of dialysis or kidney transplant in the study. The bars indicate 95% CI.

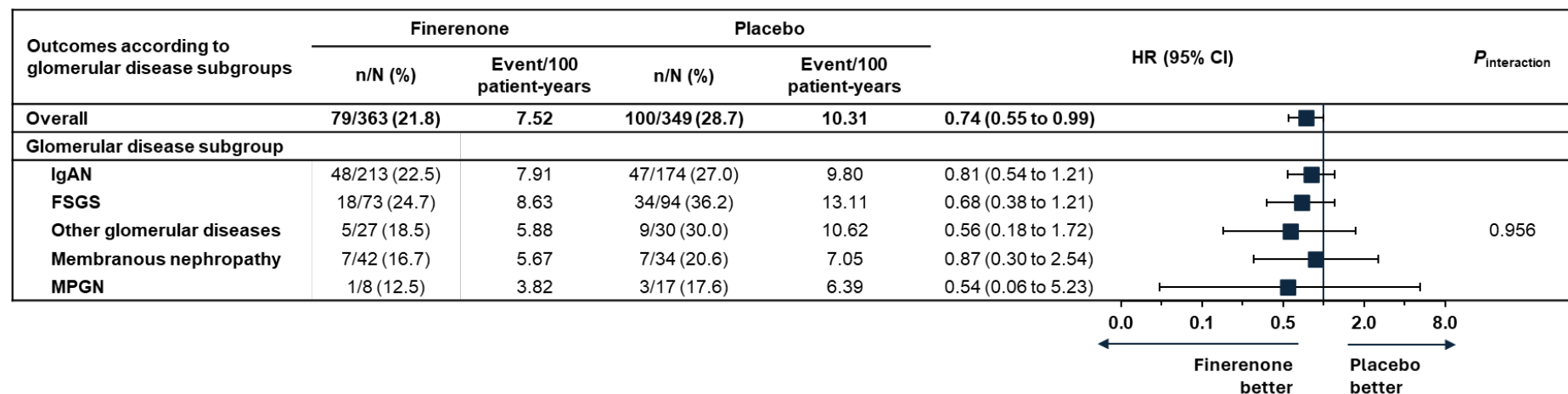
Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; FSGS, focal segmental glomerulosclerosis; IgAN, immunoglobulin A nephropathy; MPGN, membranoproliferative glomerulonephritis; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UACR, urinary albumin-to-creatinine ratio.

eFigure5. Change in UPCR From Baseline to Month 12 According to Glomerular Disease Etiology



Analyzed using a mixed model for repeated measures with fixed effects for the following factors: treatment group, visit, treatment by visit interaction, baseline SGLT2i use, and the interaction between SGLT2i use and visit, with the log-transformed baseline UPCR value as a covariate and an interaction term between the log-transformed baseline value and visit. The bars indicate 95% CI. Abbreviations: CI, confidence interval; FSGS, focal segmental glomerulosclerosis; IgAN, immunoglobulin A nephropathy; MPGN, membranoproliferative glomerulonephritis; SGLT2i, sodium-glucose cotransporter-2 inhibitor; UPCR, urinary protein-to-creatinine ratio.

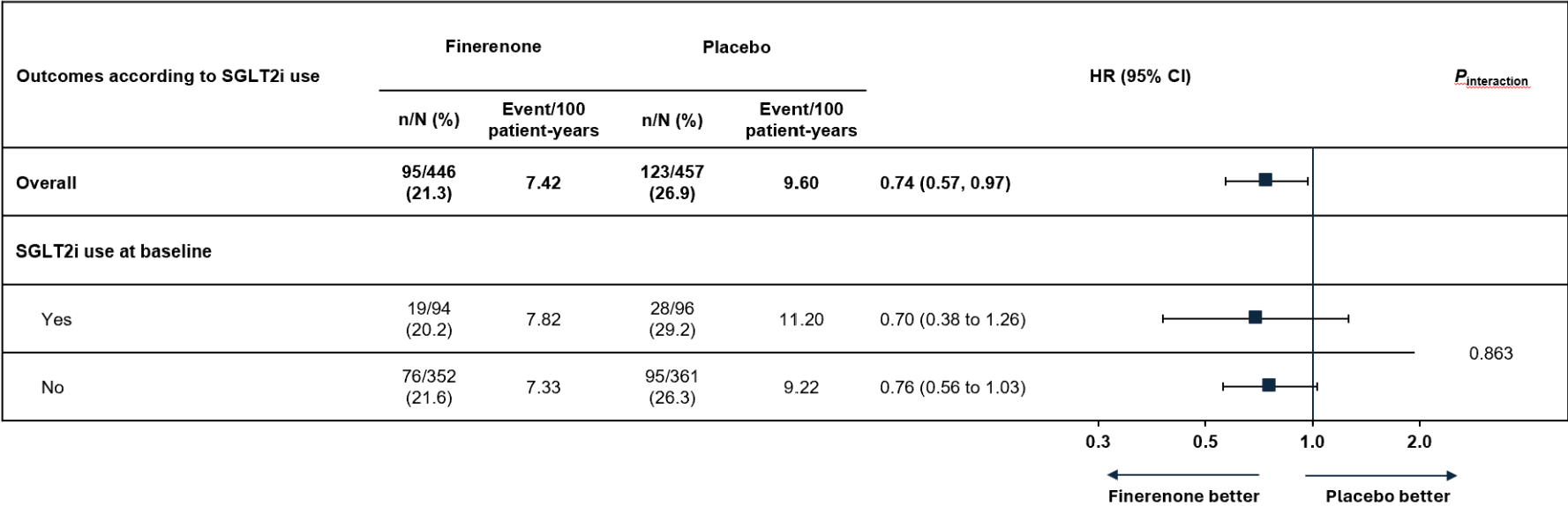
eFigure6. Subgroup Analysis for Kidney Failure or $\geq 40\%$ Decline in eGFR by Glomerular Disease Subtype in the Biopsy Cohort



Outcomes were assessed in time-to-event analyses, up to 48 months after randomization.

Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; FSGS, focal segmental glomerulosclerosis; HR, hazard ratio; IgAN, immunoglobulin A nephropathy; MPGN, membranoproliferative glomerulonephritis.

eFigure7. Subgroup Analysis for Kidney Failure or ≥40% Decline in eGFR by SGLT2i Use at Baseline



Outcomes were assessed in time-to-event analyses, up to 48 months after randomization.
Abbreviations: CI, confidence interval; eGFR, estimated glomerular filtration rate; HR, hazard ratio; SGLT2i, sodium-glucose cotransporter-2 inhibitor.